Treating Endometrial Cancer

If you've been diagnosed with endometrial cancer, your cancer care team will discuss your treatment options with you. It's important to weigh the benefits of each treatment option against the possible risks and side effects.

How is endometrial cancer treated?

The most common types of treatment for women with endometrial cancer are:

- Surgery for Endometrial Cancer
- Radiation Therapy for Endometrial Cancer
- Chemotherapy for Endometrial Cancer
- Hormone Therapy for Endometrial Cancer
- Targeted Therapy for Endometrial Cancer
- Immunotherapy for Endometrial Cancer

Common treatment approaches

Surgery is the main treatment for most women with this cancer. But in some cases, a more than 1 kind of treatment may be used. The choice of treatment depends largely on the type of cancer and stage of the disease when it's found. Other factors could play a part in choosing the best treatment plan. These include your age, your overall state of health, whether you plan to have children, and other personal considerations.

- Treatment Choices for Endometrial Cancer, by Stage

Who treats endometrial cancer?

Depending on the type and stage of the endometrial cancer, you may need more than one type of treatment. Doctors on your cancer treatment team may include:

- A **gynecologist:** a doctor who specializes in diseases of the female reproductive tract
- A **gynecologic oncologist:** a doctor who specializes in treating cancers of the female reproductive system (including surgery and chemotherapy)
- A **radiation oncologist:** a doctor who uses radiation to treat cancer
- A **medical oncologist:** a doctor who uses chemotherapy and other medicines to treat cancer

Many other specialists may be involved in your care as well, including nurses, nurse practitioners, social workers, psychologists, rehabilitation specialists, and other health professionals.

- Who Is the Cancer Care Team?

Making treatment decisions

It's important to talk with your family and treatment team about all of your treatment options, as well as their possible side effects, so you make the choice that best fits your needs. If there's anything you don't understand, ask to have it explained.

If time permits, it's often a good idea to seek a second opinion. A second opinion can give you more information and help you feel more sure of the treatment plan you choose.

- Questions to Ask About Endometrial Cancer
- Seeking a Second Opinion

Thinking about taking part in a clinical trial

Clinical trials are carefully controlled research studies that are done to get a closer look at promising new treatments or procedures. Clinical trials are one way to get state-of-the-art cancer treatment. In some cases they may be the only way to get access to newer treatments. They are also the best way for doctors to learn better methods to treat cancer.

If you would like to learn more about clinical trials that might be right for you, start by asking your doctor if your clinic or hospital conducts clinical trials.

- Clinical Trials

Considering integrative (holistic) and alternative methods

You may hear about alternative or integrative (holistic) methods to relieve symptoms or treat your cancer that your doctors haven't mentioned. These methods can include vitamins, herbs, and special diets, or other methods such as acupuncture or massage, to name a few.

Integrative methods are treatments that are used **along with** your regular medical care. **Alternative** treatments are used **instead of** standard medical treatment. Although some of these methods might be helpful in relieving symptoms or helping you feel better, many have not been proven to work. Some might even be harmful. For example, some supplements might interfere with chemotherapy.

Be sure to talk to your cancer care team about any method you are thinking about using. They can help you learn what is known (or not known) about the method, which can help you make an informed decision.

- Understanding Integrative (Holistic) Medicine

Help getting through cancer treatment

People with cancer need support and information, no matter what stage of illness they may be in. Knowing all of your options and finding the resources you need will help you make informed decisions about your care.

Whether you are thinking about treatment, getting treatment, or not being treated at all, you can still get supportive care to help with pain or other symptoms. Communicating with your cancer care team is important so you understand your diagnosis, what treatment is recommended, and ways to maintain or improve your quality of life.

Different types of programs and support services may be helpful, and they can be an important part of your care. These might include nursing or social work services, financial aid, nutritional advice, rehab, or spiritual help.

The American Cancer Society also has programs and services - including rides to treatment, lodging, and more - to help you get through treatment. Call our Cancer Knowledge Hub at 1-800-227-2345 and speak with one of our caring, trained cancer helpline specialists. Or, if you prefer, you can use our chat feature on cancer.org to connect with one of our specialists.

- Palliative Care
- Programs & Services

Choosing to stop treatment or choosing no treatment at all

For some people, when treatments have been tried and are no longer controlling the cancer, it could be time to weigh the benefits and risks of continuing to try new treatments. Whether or not you continue treatment, there are still things you can do to help maintain or improve your quality of life.

Some people, especially if the cancer is advanced, might not want to be treated at all. There are many reasons you might decide not to get cancer treatment, but it’s important to talk to your doctors as you make that decision. Remember that even if you choose not to treat the cancer, you can still get supportive care to help with pain or other symptoms.

People who have advanced cancer and who are expected to live less than 6 months may want to consider hospice care. Hospice care is designed to provide the best possible quality of life for people who are near the end of life. You and your family are encouraged to talk with your doctor or a member of your supportive care team about hospice care options, which include hospice care at home, a special hospice center, or other health care locations. Nursing care and special equipment can make staying at home a workable option for many families.

- If Cancer Treatments Stop Working
- Hospice Care

The treatment information given here is not official policy of the American Cancer Society and is not intended as medical advice to replace the expertise and judgment of your cancer care team. It is intended to help you and your family make informed decisions, together with your doctor. Your doctor may have reasons for suggesting a treatment plan different from these general treatment options. Don't hesitate to ask your cancer care team any questions you may have about your treatment options.

Surgery for Endometrial Cancer

Surgery is used for uterine cancer for several reasons:

- **For staging**, to evaluate if and where the cancer has spread outside of the uterus.
- **For treatment**, to remove all cancer in the abdomen and pelvis.

- **For symptom management**, if the cancer has spread to distant parts of the body.

Surgery usually includes a total hysterectomy (removal of the uterus and cervix), bilateral salpingo-oophorectomy (removal of both fallopian tubes and ovaries), and lymphadenectomy (assessment and removal of certain lymph nodes).

Typically, peritoneal lavage is also done during surgery. The abdominal and pelvic cavities are “washed” with salt water (saline) which is then collected and sent to the lab to see if it contains cancer cells. The results of testing the fluid from the peritoneal lavage does not affect the current staging, but both FIGO and the American Joint Committee on Cancer (AJCC) recommend collecting the sample.

During surgery, the surgeon will also inspect the peritoneum (membrane around the abdominal and pelvic cavity) and will biopsy any suspicious areas to check for cancer cells.If endometrial cancer cells are found, the cancer’s surgical stage may change, and the next steps of treatment could be affected.

For surgical treatment, the goal is to remove as much cancer as possible. This is called **debulking** or **cytoreduction**.

If the surgery is being done to manage symptoms, the procedure will target specific symptoms, such as blocked bowels.

What is a hysterectomy?

Hysterectomy is surgery to remove your uterus (womb).

Depending on the type of cancer you have and whether it has spread to nearby organs, you might also have your ovaries (oophorectomy) and fallopian tubes (salpingectomy) removed. You might also need to have nearby lymph nodes (lymphadenectomy) removed to see if the cancer has spread to them.

Types of hysterectomy

- **Total hysterectomy** – Removal of the uterus and cervix.
- **Subtotal (partial) hysterectomy** – Removal of the uterus, leaving the cervix intact.
- **Radical hysterectomy** – Removal of the uterus, cervix, upper vagina, and surrounding tissues.
- **Hysterectomy with bilateral salpingo-oophorectomy** – Removal of the uterus, cervix, both ovaries, and fallopian tubes (also called a **radical hysterectomy**).

The standard type of hysterectomy for uterine cancer is a total hysterectomy. However, if the endometrial cancer is suspected to have spread to the cervix, a radical hysterectomy could be done to make sure all the cancer is removed.

Surgical approaches for hysterectomy

Hysterectomies can be done in different ways (approaches). Your surgeon will talk through options with you and suggest what approach they think is best for you.

Abdominal hysterectomy (laparotomy)

This surgery is done through an incision (cut) in the front of your abdomen (belly). This lets your surgeon look directly at the cancer area. Your uterus and other organs can then be removed through this incision.

Also called **open surgery**, this approach helps your surgeon remove as much of your cancer as possible. It also decreases the risk of injury to organs near the uterus, such as your bladder and bowel. It often takes longer to recover from open surgery than other methods.

Vaginal hysterectomy

This surgery is done through your vagina. An incision is made at the top of your vagina, and your uterus is removed.

Vaginal hysterectomy is seldom used to treat cancer because it doesn't let the surgeon clearly see all the cancer. But it might be an option if you have health problems that make it too risky for you to have other types of hysterectomy.

Laparoscopic hysterectomy (minimally invasive surgery)

This surgery only requires small incisions. A thin tube called a **laparoscope** is put through an incision. The laparoscope has a tiny video camera at the end, which lets the surgeon look at the inside of your abdomen (belly) and pelvis.

Small surgical instruments are then inserted through the laparoscope or other incision. The surgeon uses the instruments to cut around the uterus and other organs that need to be removed. Your uterus can often be removed through your vagina.

Laparoscopic hysterectomy usually causes less pain and blood loss because the incisions are smaller. Recovery time is usually shorter than from an abdominal hysterectomy.

Robotic-assisted laparoscopic hysterectomy

Robotic technology is sometimes used during laparoscopic hysterectomies. For this, the surgeon sits at a control panel in the operating room and operates through special tools attached to robotic arms . This also only requires small incisions.

Pain, blood loss, and recovery are much the same as with laparoscopic hysterectomy. Ask your surgeon how much experience they have with robotic-assisted surgery and how their other patients have done.

Other surgery done with hysterectomy

Bilateral salpingo-oophorectomy

A **bilateral salpingo-oophorectomy (BSO)** removes both ovaries and fallopian tubes.

It is usually done at the same time as the hysterectomy to treat endometrial cancers.

Having both ovaries removed means that you'll go into menopause if you haven't done

so already. If you have endometrial cancer and have not yet gone into menopause it may be safe to keep your ovaries if you have a low-stage, non-aggressive endometrial cancer. Talk with your surgeon about this decision. When ovaries are removed there might be a lower chance of the cancer coming back, but for patients who keep their ovaries, there doesn’t seem to be an increased risk of dying from endometrial cancer.

Lymph node surgery

Sentinel lymph node dissection (SLND) and lymphatic mapping may be used in early-stage endometrial cancer if imaging tests don't clearly show signs that cancer has spread to the lymph nodes in your pelvis. For this procedure, a blue or green dye is injected into the cervix. The surgeon then looks for the lymph nodes that turn blue or green (from the dye). This is called **lymphatic mapping**. These lymph nodes are the first ones the cancer would drain into and are called **sentinel lymph nodes**. They are removed and tested to see if they have cancer cells. This is a sentinel lymph node dissection. If they do have cancer cells, more lymph nodes may be taken out.

If there are no cancer cells in the sentinel nodes, no more nodes are removed. This procedure is usually done at the same time as surgery to remove the uterus (hysterectomy).

If a sentinel lymph node is not identified on both sides, then this is a failure of mapping. In this situation the lymph nodes should be removed from each side that failed to map.

If imaging tests done before surgery show enlarged lymph nodes (indicating the cancer might have spread) the lymph nodes in specific areas from the pelvis and from the area next to the aorta will be removed during the hysterectomy. They are then tested to see if they contain cancer cells that have spread from the endometrial tumor. This information is part of finding the cancer’s surgical stage.

Omentum biopsy

The omentum is a flat layer of fat on the surface of the abdominal organs. A sample from this structure is taken during surgery for aggressive types of endometrial cancer.

Recovery after a hysterectomy for uterine cancer

Your recovery will depend on the type of hysterectomy you had and what surgical approach was used.

The hospital stay after an **abdominal hysterectomy (laparotomy)** is usually 2 to 7 days. Complete recovery can take 4 to 6 weeks.

If the hysterectomy is done by a **laparoscopic** or robotic or **vaginal** approach, usually you will be able to leave the hospital on the same day or the next day. Full recovery takes about 3 to 4 weeks.

Possible side effects of hysterectomy

Complications of these surgeries are not common and depend on the surgical approach. They can include nerve or vessel damage, excessive bleeding, wound infection, and damage to nearby tissues (the urinary and intestinal systems).

You might also be at higher risk for blood clots. That’s why you will be encouraged to get out of bed as soon as possible. You may also be given stockings or a device to compress your legs and help move blood back out of your legs.

If you have many pelvic lymph nodes removed, you might be at risk for swelling in your legs (lymphedema[1]). This risk is higher if you have radiation after surgery.

A radical hysterectomy can affect the nerves that control the bladder, so a catheter is used to drain urine after surgery. It may be needed for a few days or up to 2 weeks. If the bladder hasn’t recovered completely when the catheter is removed, it may need to be put back in. Or, you can be shown how to put in a catheter several times a day to empty your bladder. Over time, your bladder function should return .

Long-term side effects of uterine surgery

Any hysterectomy causes infertility (you won't be able to get pregnant).

Removing the ovaries will cause menopause right away. This can lead to symptoms like hot flashes, night sweats, and vaginal dryness. It can lead to osteoporosis and increased risk for heart disease, which impacts all post-menopausal women.

Removing lymph nodes in the pelvis can lead to a build-up of fluid in the legs and genitals. This can become a life-long problem called lymphedema[2]. It's more likely if radiation is given after surgery.

Surgery and menopausal symptoms can also affect your sex life. For more, see How Cancer Surgery Can Affect Sex for Women[3].

Talk with your treatment team about side effects you might have right after surgery and later on. There might be things you can do to help prevent side effects. Know what to expect so you can get help right away.

More information about surgery

For more general information about surgery as a treatment for cancer, see Cancer Surgery[4].

To learn about some of the side effects listed here and how to manage them, see Managing Cancer-related Side Effects[5].

Hyperlinks

1. www.cancer.org/cancer/managing-cancer/side-effects/swelling/lymphedema.html
2. www.cancer.org/cancer/managing-cancer/side-effects/swelling/lymphedema.html
3. www.cancer.org/cancer/managing-cancer/side-effects/sexual-side-effects/pelvic-surgery.html
4. www.cancer.org/cancer/managing-cancer/treatment-types/surgery.html
5. www.cancer.org/cancer/managing-cancer/side-effects.html

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Radiation Therapy for Endometrial Cancer

Radiation therapy can be given in 2 ways to treat endometrial cancer:

- By putting radioactive materials inside the body. This is called **internal radiation therapy** or **brachytherapy**.
- By using a machine that focuses beams of radiation at the tumor, much like having an x-ray. This is called **external beam radiation therapy**.

Sometimes, both brachytherapy and external beam radiation therapy are used. The external beam radiation is usually given first, followed by brachytherapy. The stage[1] and grade of the cancer are used to help decide what areas need to be treated with radiation therapy and which types of radiation are used.

When is radiation used to treat endometrial cancer?

Radiation most often is used after surgery to treat uterine cancer. It can kill many cancer cells that may still be in the treated area. If your treatment plan includes radiation after surgery, you will be given time to heal before starting radiation. Often, at least 4 to 6 weeks are needed. Less often, radiation might be given before surgery to help shrink a tumor so it's easier to remove.

Those who are not healthy enough for surgery may get radiation as their main treatment.

Brachytherapy for endometrial cancer

After the uterus and cervix are removed, the upper part of the vagina may be treated

with brachytherapy. This is called **vaginal brachytherapy**. A radioactive material is put into a cylinder (an applicator) and the cylinder is put into the vagina. (It feels a lot like a snug tampon.)

The size of the cylinder and how much radiation is in it might vary. With brachytherapy,

the radiation mainly affects the area of the vagina in contact with the cylinder. Nearby structures like the bladder and rectum get less radiation exposure.

This procedure is done in the radiation therapy area of a hospital or a radiation

treatment center. Two types of brachytherapy are used for endometrial cancer: low-dose rate (LDR) and high-dose rate (HDR).

Low-dose rate brachytherapy

The applicator with the radiation source in it is left in place for up to 4 days. The patient needs to be still to keep the applicator from moving during treatment, so this requires a hospital stay during treatment. Because the patient must not move, this form of brachytherapy carries a risk of serious blood clots in the legs (called **deep venous thrombosis or DVT**). LDR isn't commonly used in the United States.

High-dose rate brachytherapy

The radiation given is stronger. Each treatment takes a very short time (usually less than an hour), and the radiation is only in for 10 to 20 minutes. The applicator is only in place while the treatment is done. You will be able to go home the same day. For endometrial cancer, HDR brachytherapy might be given weekly or even daily for at least 3 doses.

The most common side effect is a change in the lining of the vagina. (Called **radiation vaginitis**, this is discussed in the side effects section.) If needed, pain medicines can be used to help you be more comfortable while the applicator is in.

External beam radiation therapy for endometrial cancer

In this type of treatment the radiation is delivered from a source outside of the body.

External beam radiation therapy is often given 5 days a week for 4 to 6 weeks. The skin covering the treatment area is carefully marked with ink or tiny tattoos. A special mold of the pelvis and lower back is custom-made to make sure you are in the exact same position for each treatment. Each treatment takes less than a half-hour, but daily visits to the radiation center are usually needed.

Sometimes chemotherapy is given along with the radiation to help the radiation work better. This is called **chemoradiation**.

Side effects of radiation therapy for endometrial cancer

Short-term side effects

Severe fatigue, which might not start until about 2 weeks after treatment begins, is also common. Diarrhea is common but usually can be controlled with over-the-counter medicines. Nausea and vomiting may occur but can be treated with medicine. These side effects are more common with external beam radiation than with brachytherapy.

Side effects tend to be worse when chemotherapy is given with radiation.

- **Skin changes**, which can range from mild redness to peeling and blistering, are quite common. The skin may be more vulnerable to infection, so care must be taken to clean and protect the area exposed to radiation. Sometimes, as it heals, the skin in the treated area becomes darker or less flexible (harder).
- **Irritation to the bladder**, called **radiation cystitis**, can result in discomfort and problems urinating, blood in the urine, and an urge to urinate often.
- Irritation in the intestine
- Rectal irritation or bleeding is called **radiation proctitis**. It's sometimes treated with enemas that contain a steroid (like hydrocortisone) or suppositories that contain an anti-inflammatory.
- **Vaginal irritation** leading to discomfort and drainage (a discharge). This is called **radiation vaginitis**. If it occurs, the doctor may recommend douching with a dilute solution of hydrogen peroxide. When the irritation is severe, open sores can develop in the vagina, which may need to be treated with certain creams.

- **Low blood counts**, causing anemia (low red blood cells) and leukopenia (low white blood cells). The blood counts usually return to normal within a few weeks after radiation is stopped.

Long-term side effects

Vaginal dryness. Vaginal dryness is more common after vaginal brachytherapy than after pelvic radiation therapy. In some cases scar tissue can form in the vagina. The scar tissue can make the vagina shorter or more narrow (called **vaginal stenosis**), which can make sex (vaginal penetration) painful. Ask your doctor about this if you are bothered by these problems. You can also find some helpful information in How Radiation Therapy Can Affect Sex for Women[2].

Premature menopause. Pelvic radiation can damage the ovaries, resulting in premature menopause. This is not an issue for most treated for endometrial cancer because they usually have already gone through menopause, either naturally or as a result of surgery to treat the cancer (hysterectomy and removal of the ovaries).

Lymphedema. Pelvic radiation therapy can also lead to blockages that keep fluid from draining out of the leg. This can lead to severe swelling, called **lymphedema**. Lymphedema is a long-term side effect; it doesn't go away after radiation is stopped. In fact, it might not start for several months or even years after treatment ends. This side effect is more common if pelvic lymph nodes were removed during surgery (lymph node dissection) to stage the cancer. There are specialized physical therapists who can help treat this. It's important to start treatment right away if you develop it. To learn more, see Lymphedema[3].

Weakened bones. Radiation to the pelvis can **weaken the bones**, leading to fractures of the hips or pelvic bones. It's important that those who have had endometrial cancer contact their doctor right away if they have pelvic pain. Such pain might be caused by a fracture, recurrent cancer (cancer that's come back after treatment), or other serious conditions.

Bladder and bowel problems. Pelvic radiation can also lead to long-term problems with the bladder (radiation cystitis) or bowel (radiation proctitis). Rarely, radiation damage to the bowel can cause a **blockage** (called **obstruction**) or cause an abnormal connection between the bowel and the vagina or outside skin (called a **fistula**). These conditions may need to be treated with surgery.

If you are having side effects from radiation, be sure to tell someone on your cancer care team. There are things you can do to get relief from these symptoms or to prevent them from happening. It's very important to talk about what to expect, and continue to talk about what's changing or has changed in your life as you go through procedures, treatments, and follow-up care. **Don't assume your doctor or nurse will ask about any concerns you have about sexuality.** Remember, if they don't know you're having a problem, they can't help you manage it.

More information about radiation therapy

To learn more about how radiation is used to treat cancer, see Radiation Therapy[4].

To learn about some of the side effects listed here and how to manage them, see Managing Cancer-related Side Effects[5].

Hyperlinks

1. www.cancer.org/cancer/types/endometrial-cancer/detection-diagnosis-staging/staging.html
2. www.cancer.org/cancer/managing-cancer/side-effects/sexual-side-effects/pelvic-radiation.html
3. www.cancer.org/cancer/managing-cancer/side-effects/swelling/lymphedema.html
4. www.cancer.org/cancer/managing-cancer/treatment-types/radiation.html
5. www.cancer.org/cancer/managing-cancer/side-effects.html

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Chemotherapy for Endometrial Cancer

Chemotherapy (Chemo) is the use of drugs to destroy cancer cells, usually by keeping the cancer cells from growing, dividing, and making more cells. When recommended for endometrial cancer treatment, chemotherapy usually is given after surgery. Chemotherapy is also considered if the endometrial cancer returns after the initial treatment.

Chemo drugs used to treat endometrial cancer

Chemo drugs used to treat endometrial cancer may include:

- Paclitaxel (Taxol®)
- Carboplatin
- Cisplatin
- Doxorubicin (Adriamycin®) or liposomal doxorubicin (Doxil®)
- Bevacizumab (Avastin)
- Docetaxel (Taxotere®)
- Gemcitabine

Most often, 2 or more drugs are combined for treatment. The most common combinations include carboplatin/paclitaxel and cisplatin/doxorubicin.

For uterine carcinosarcoma, the chemo drug ifosfamide (Ifex®) might be used, either alone or along with either cisplatin or paclitaxel.

When and how are chemo drugs used to treat endometrial cancer

A chemotherapy regimen, or schedule, usually consists of a specific number of cycles given over a set period of time. A patient may receive one drug at a time or a combination of different drugs given at the same time.

The goal of chemotherapy is to destroy cancer cells left after surgery or to shrink the cancer and slow the tumor's growth if it comes back or has spread to other parts of the body. Although chemotherapy can be given orally, most drugs used to treat endometrial cancer are given by IV. IV chemotherapy is either injected directly into a vein or through a catheter, which is a thin tube inserted into a vein.

Sometimes chemo is given for a few cycles, followed by radiation. Then chemo is given again. This is called **sandwich therapy**. Another treatment option is to give chemo with radiation (called **chemoradiation**). The chemo can help the radiation work better, but it can be harder on the patient because the combination causes more side effects. The chemo given in this situation is usually cisplatin.

Side effects of chemotherapy for endometrial cancer

These drugs kill endometrial cancer cells but can also damage some normal cells, which in turn causes side effects. Side effects of chemotherapy depend on the drugs used, the amount taken, and how long treatment is given. Common side effects include:

- Nausea and vomiting
- Loss of appetite
- Mouth sores
- Neuropathy
- Hair loss
- Rarely, some drugs cause some hearing loss

Also, most chemo drugs can damage the blood-producing cells of the bone marrow. This can result in low blood cell counts[1], such as:

- Low white blood cells, which increases the risk of infection
- Low platelet counts, which can cause bleeding **or** bruising after minor cuts or injuries
- Low red blood cells (anemia), which can cause problems like fatigue and shortness of breath

Heart damage

Most of the side effects of chemotherapy get better over time when treatment ends, but some can last a long time. For instance, doxorubicin can damage the heart muscle over time. The chance of heart damage goes up as the total dose of the drug goes up, so doctors limit how much doxorubicin a person will get.

Kidney damage

Cisplatin can cause kidney damage, so you'll be given lots of IV fluids before and after chemo to help protect the kidneys. Both cisplatin and paclitaxel can cause nerve damage (called neuropathy[2]). This can lead to numbness, tingling, or even pain in the hands and feet.

Ifosfamide can injure the lining of the bladder, causing it to bleed (called **hemorrhagic cystitis**). To prevent this, you might be given large amounts of IV fluids and a drug called **mesna** along with the chemo.

Infertility

Other possible side effects of chemotherapy for endometrial cancer include infertility[3] (the inability to become pregnant in the future) and early menopause, if the patient has not already had a hysterectomy. Talk with your doctor **before treatment starts** if you want to save your fertility.

Before starting chemotherapy, be sure to discuss the drugs and their possible side effects with your health care team.

If you have side effects while getting chemotherapy, remember that there are ways to prevent or treat most of them. For instance, there are many drugs that can help prevent or reduce nausea and vomiting. Be sure to tell your health care team about any side effects you have. Treating them right away can often keep them from getting worse.

More information about chemotherapy

For more general information about how chemotherapy is used to treat cancer, see Chemotherapy[4].

To learn about some of the side effects listed here and how to manage them, see Managing Cancer-related Side Effects[5].

Hyperlinks

1. www.cancer.org/cancer/managing-cancer/side-effects/low-blood-counts.html

2. www.cancer.org/cancer/managing-cancer/side-effects/pain/peripheral-neuropathy.html
3. www.cancer.org/cancer/managing-cancer/side-effects/fertility.html
4. www.cancer.org/cancer/managing-cancer/treatment-types/chemotherapy.html
5. www.cancer.org/cancer/managing-cancer/side-effects.html

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Hormone Therapy for Endometrial Cancer

This type of treatment uses hormones or hormone-blocking drugs to treat cancer. It’s not the same as the hormone therapy given to ease the symptoms of menopause (menopausal hormone therapy[1]). Hormone therapy can be used to treat advanced endometrial cancer (stage III or IV) or cancer that has come back after treatment (recurred). It is also used for early-stage cancer when surgery is not an option, or in patients who wish to keep the uterus to save their fertility. It’s usually also considered for cancer that is lower-grade, low volume, and slow growing.

Hormone treatment for endometrial cancer

Hormone treatment for endometrial cancercan include:

- Progestins (this is the main hormone treatment used)
- Aromatase inhibitors (AIs)
- Tamoxifen
- Fulvestrant
- CDK 4/6 inhibitors

At this time, no one type of hormone treatment has been found to be the best for endometrial cancer.

Progestins

The main hormone treatment for endometrial cancer uses progesterone or drugs like it (called **progestins**).

The progestins used to manage endometrial cancer are:

- Medroxyprogesterone acetate (**Provera**®), which can be given as an injection or as a pill
- Megestrol acetate (**Megace**®), which can be given as a pill or liquid
- **Levonorgestrel intrauterine device (IUD)** which is placed inside the uterus and releases levonorgestrel daily for many years. This is the preferred hormonal treatment for patients with endometrial cancer that has not spread beyond the

uterus and who are unable to have surgery or who choose not to have surgery because of plans for a future pregnancy.

Side effects of progestins can include:

- Hot flashes
- Night sweats
- Weight gain (from fluid retention and an increased appetite)
- Worsening of depression
- Increased blood sugar levels in women with diabetes
- Serious blood clots (this is rare)

Aromatase inhibitors (AIs)

Before menopause, most estrogen is made by the ovaries. Even after the ovaries are removed (or are not working), estrogen is still made in fat tissue by an enzyme called **aromatase**. Aromatase inhibitors work by preventing aromatase from making estrogen.

Examples of aromatase inhibitors are:

- **Letrozole** (Femara®)
- **Anastrozole** (Arimidex®)
- **Exemestane** (Aromasin®)

These drugs are most often used to treat breast cancer, but can be helpful in treating endometrial cancer, too. These drugs are taken as pills, typically once a day.

Common side effects of aromatase inhibitors can include:

- Joint and muscle pain
- Mood changes
- Vaginal dryness
- Hot flashes

If taken for a long time (years), these drugs also can weaken bones, sometimes leading to osteoporosis. These drugs are still being studied for how to best use them to treat endometrial cancer.

Tamoxifen

Tamoxifen is an anti-estrogen drug that can be used to treat advanced or recurrent endometrial cancer. Alternating a progestin and tamoxifen seems to work well and be better tolerated than giving a progestin alone.

Tamoxifen blocks estrogen from connecting to the cancer cells and telling them to grow and divide. It is a pill taken by mouth, daily.

Common side effects of tamoxifen include:

- Hot flashes
- Vaginal dryness

Rare but serious side effects include blood clots in the legs (called **deep vein thrombosis or DVT**) or blood clots in the lungs (called **pulmonary emboli or PE)**.

Fulvestrant

Fulvestrant is an anti-estrogen drug also called a selective estrogen receptor degrader (SERD)**,** that can be used to treat advanced or recurrent endometrial cancer. It is given as 2 injections into the buttocks. For the first month, the 2 shots are given 2 weeks apart. After that, they are given once a month.

Common side effects of fulvestrant include:

- Hot flashes and/or night sweats
- Headache
- Nausea
- Feeling tired
- Loss of appetite
- Muscle, joint, or bone pain

CDK 4/6 inhibitors

Some endometrial cancers are hormone (estrogen or progesterone) receptor-positive. For these uterine cancers, CDK 4/6 inhibitors may be a treatment option.

Ribociclib (Kisqali) and **abemaciclib** (Verzenio) are drugs that block proteins in the cell called **cyclin-dependent kinases (CDKs)**, particularly CDK4 and CDK6. Blocking these proteins in hormone receptor-positive uterine cancer cells helps stop the cells from dividing. This can slow cancer growth. Ribociclib or abemaciclib can be given with an AI (letrozole). These drugs are pills typically taken once or twice a day.

The most common **side effects of CDK4/6 inhibitors** include:

- Low blood cell counts (low white cell counts can increase the risk of infection)
- Fatigue

Less common side effects include:

- Nausea and vomiting
- Mouth sores
- Hair loss
- Diarrhea
- Headache

A rare but possible life-threatening side effect is inflammation of the lungs, also called **interstitial lung disease or pneumonitis**.

More information about hormone therapy

To learn more about how hormone therapy is used to treat cancer, see Hormone Therapy[2].

To learn about some of the side effects listed here and how to manage them, see Managing Cancer-related Side Effects[3].

Hyperlinks

1. www.cancer.org/cancer/risk-prevention/medical-treatments/menopausal-hormone-replacement-therapy-and-cancer-risk.html
2. www.cancer.org/cancer/managing-cancer/treatment-types/hormone-therapy.html
3. www.cancer.org/cancer/managing-cancer/side-effects.html

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Targeted Therapy for Endometrial Cancer

Targeted therapy drugs are made to attack certain changes in the cancer cells. These drugs work differently from standard chemotherapy (chemo). They tend to have different (and sometimes less severe) side effects.

Targeted therapy drugs are mostly used to treat high-risk endometrial cancers and those that have spread (metastasized) or come back (recurred) after treatment.

Trastuzumab (Herceptin)

Trastuzumab (Herceptin) is a monoclonal antibody[1], a man-made version of an immune system protein, which targets HER2 (a protein that plays a role in cell growth and division). Adding trastuzumab to chemo is a treatment option for advanced or recurrent HER2-positive endometrial cancer, specifically uterine serous carcinoma and carcinosarcoma. The chemo given with trastuzumab is usually carboplatin and paclitaxel. Trastuzumab is given as an infusion into the vein (IV) every 3 weeks along with chemo.

Common side effects of trastuzumab include:

- Fever and chills
- Weakness
- Nausea and vomiting
- Cough
- Diarrhea
- Headache

These side effects occur less often after the first dose. A rare but serious side effect is heart damage. Before starting treatment with this drug, your doctor will test your heart function with an echocardiogram or a MUGA scan.

Fam-trastuzumab deruxtecan-nxki (Enhertu)

Fam-trastuzumab deruxtecan (Enhertu) is an antibody-drug conjugate (ADC), which is a HER2 antibody connected to a chemo drug, deruxtecan. When the ADC is infused into the body, the antibody acts like a homing device, attaching to the HER2 protein on cancer cells and bringing the chemo directly to them.

It can be a treatment option for advanced or recurrent HER2-positive endometrial cancer, after at least 1 other drug has been tried. This drug is given as an infusion into a vein (IV) every 3 weeks.

Common side effects of fam-trastuzumab deruxtecan-nxki include:

- Low blood cell counts (which can increase a person’s risk of infections and bleeding),
- Nausea and vomiting
- Diarrhea or constipation
- Loss of appetite
- Fever
- Feeling tired
- Hair loss

A rare but serious side effect is lung disease and heart damage. It’s very important to let your doctor or nurse know right away if you’re having symptoms such as coughing, wheezing, trouble breathing, or fever. Before starting treatment with this drug, your doctor will test your heart function with an echocardiogram or a MUGA scan.

Lenvatinib (Lenvima)

Lenvatinib (Lenvima) is a **kinase inhibitor**. It helps block tumors from forming new blood vessels, as well as targets some of the proteins in cancer cells that normally help them grow. It is used along with the immunotherapy drug pembrolizumab (Keytruda) to treat some advanced uterine cancers, typically after treatment with carboplatin (a chemo drug) has been tried. Lenvatinib is taken as capsules once a day.

Common side effects of lenvatinib include:

- Diarrhea
- Fatigue
- Joint or muscle pain
- Loss of appetite
- Nausea and vomiting
- Mouth sores
- Weight loss
- High blood pressure
- Swelling in the arms or legs

Less common but more serious side effects can include serious bleeding, blood clots, very high blood pressure, severe diarrhea, holes forming in the intestines, and kidney, liver, or heart failure.

Bevacizumab (Avastin)

For cancers to grow and spread, they need to make new blood vessels to nourish themselves (a process called **angiogenesis**). Bevacizumab belongs to a class of drugs called **angiogenesis inhibitors**. This drug attaches to a protein called **VEGF** (which signals new blood vessels to form) and slows or stops cancer growth.

Bevacizumab can be given for treatment of advanced or recurrent endometrial cancer. It is often given with chemotherapy as the first treatment option, but it can also be given alone after other drug treatments have been tried. This drug is given as an infusion into a vein (IV) every 2 to 3 weeks.

Common side effects of bevacizumab include:

- High blood pressure

- Tiredness
- Bleeding
- Low white blood cell counts
- Headaches
- Mouth sores
- Loss of appetite
- Diarrhea

Rare, but serious side effects include blood clots, severe bleeding, slow wound healing, holes forming in the colon (perforations), and the formation of abnormal connections between the bowel and the skin or bladder (fistulas). If a perforation or fistula forms, it can lead to severe infection and surgery may be needed.

mTOR inhibitors

These drugs block mTOR, a protein in cells that normally helps them grow and divide. They may also stop tumors from developing new blood vessels, which can help limit their growth.

Everolimus (Afinitor)

Everolimus is usually given with letrozole (aromatase inhibitor) for patients with lower grade, low volume, or slow-growing endometrial cancer that has spread beyond the pelvis or has recurred. It is a pill taken once a day.

Common side effects of everolimus include:

- Mouth sores
- Rash
- Diarrhea
- Nausea
- Feeling weak or tired
- Low blood counts
- Shortness of breath
- Cough

Everolimus can also increase blood lipids (cholesterol and triglycerides) and blood sugars. It can also increase your risk of serious infections, so your doctor will watch you closely.

Temsirolimus (Torisel)

Temsirolimus is given alone for advanced endometrial cancer that has already progressed while the patient took another drug. It is given as an intravenous (IV) infusion, typically once a week.

Common side effects of temsirolimus include:

- Skin rash
- Weakness
- Mouth sores
- Diarrhea
- Nausea
- Loss of appetite
- Fluid build-up in the face or legs
- Increases in blood sugar and cholesterol levels

Cabozantinib (Cabometyx)

Cabozantinib belongs to a class of drugs called **multikinase inhibitors**, because they can block several different kinase proteins. These drugs work in two main ways:

- They help block tumors from forming new blood vessels which the tumors need to grow.

- They target some of the proteins made by cancer cells that normally help them grow.

Cabozantinib are tablets taken once daily. It is given alone for advanced endometrial cancer that has progressed while the patient took another drug.

Common side effects of cabozantinib include:

- Fatigue
- Rash
- Loss of appetite
- Diarrhea
- Nausea
- High blood pressure
- Hand foot syndrome (redness, pain, swelling, or blisters on the palms of the hands or soles of the feet)

Rare, but serious side effects include severe bleeding and holes in the intestine.

NTRK inhibitors

A very small number of endometrial cancers have changes in one of the *NTRK* genes, called ***NTRK*** **gene fusions. Cells with these gene changes make abnormal TRK proteins, which can lead to abnormal cell growth and cancer.**

Larotrectinib (Vitrakvi) or **entrectinib** (Rozlytrek) are NTRK inhibitors. NTRK inhibitors target and disable the proteins made by the *NTRK* genes. This drug can be used for advanced endometrial cancer.

These drugs are pills taken once or twice daily.

Common side effects of NTRK inhibitors include:

- Abnormal liver test results
- Decreased white blood cell and red blood cells
- Muscle and joint pain
- Tiredness
- Diarrhea or constipation
- Nausea and vomiting
- Stomach pain

Rare but serious side effects can include mental changes, such as confusion, changes in mood, and changes in sleep; liver damage; changes in heart rhythm and/or function; vision changes; and harm to a fetus.

More information about targeted therapy

To learn more about how targeted drugs are used to treat cancer, see Targeted Cancer Therapy[2].

To learn about some of the side effects listed here and how to manage them, see Managing Cancer-related Side Effects[3].

Hyperlinks

1. www.cancer.org/cancer/managing-cancer/treatment-types/immunotherapy/monoclonal-antibodies.html
2. www.cancer.org/cancer/managing-cancer/treatment-types/targeted-therapy.html
3. www.cancer.org/cancer/managing-cancer/side-effects.html

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Immunotherapy for Endometrial Cancer

An important part of your immune system is its ability to keep itself from attacking the

body's normal cells. To do this, it uses “checkpoint” proteins on immune cells, which act like switches that need to be turned on (or off) to start an immune response. Cancer cells sometimes use these checkpoints to avoid being attacked by the immune system.

Drugs that target these checkpoints (called **immune checkpoint inhibitors**) can be

used to treat some endometrial cancers.

Immunotherapy is treatment with drugs that help a person’s immune system better recognize and kill cancer cells. Immunotherapy drugs used to treat endometrial cancer are immune checkpoint inhibitors.

Using PD-1 inhibitors to treat endometrial cancer

Pembrolizumab (Keytruda), dostarlimab (Jemperli), and nivolumab (Opdivo) are drugs that target PD-1, a protein on T cells. PD-1 normally helps keep T cells from attacking other cells in the body (including some cancer cells). By blocking PD-1, these drugs boost the immune response against cancer cells. This can shrink some tumors or slow their growth.

Pembrolizumab or **dostarlimab** can be given along with chemotherapy as an initial treatment option for advanced or recurrent endometrial cancer. The chemo given with the PD-1 inhibitor is usually carboplatin and paclitaxel.

Pembrolizumab or dostarlimab or **nivolumab** can be given alone after other drug treatments have been tried and if the cancer cells have been tested and found to have a high level of **microsatellite instability (MSI-H)** or a **defect in a mismatch repair gene (dMMR).** Pembrolizumab can also be given alone after other drug treatments have been tried if the cancer cells have a **high tumor mutational burden (TMB-H)**, meaning the cells have many gene mutation

Pembrolizumab can also be used along with the targeted drug lenvatinib (Lenvima) to treat advanced endometrial cancers that are not MMR deficient (dMMR) or MSI high (MSI-H), typically after at least one other drug treatment has been tried.

These drug are given as an intravenous (IV) infusion, typically once every few weeks.

Using PDL-1 inhibitors to treat endometrial cancer

Durvalumab (Imfinzi) and **avelumab (Bavencio)** are drugs that target PDL-1, a protein related to PD-1 that is found on some tumor cells and immune cells. Blocking this protein can help boost the immune response against cancer cells. This can shrink some tumors or slow their growth.

Durvalumab can be given along with chemotherapy as an initial treatment option for advanced or recurrent endometrial cancer that has a defect in a mismatch repair gene (dMMR). The chemo given with the PDL-1 inhibitor is usually carboplatin and paclitaxel.

Avelumab can be given alone after other drug treatments have been tried and if the cancer cells have been tested and found to have a high level of **microsatellite instability (MSI-H)** or a **defect in a mismatch repair gene (dMMR).**

These drugs are given as an intravenous (IV) infusion, typically once every 2 or 4 weeks.

Possible side effects of checkpoint inhibitors include:

- Feeling tired or weak
- Fever
- Cough
- Nausea
- Itching
- Skin rash
- Loss of appetite
- Muscle or joint pain
- Shortness of breath
- Constipation or diarrhea

Other, more serious side effects occur less often but can include:

Infusion reactions: Some people might have an infusion reaction while getting one of these drugs. This is like an allergic reaction, and can include fever, chills, flushing of the face, rash, itchy skin, feeling dizzy, wheezing, and trouble breathing. It's important to tell your doctor or nurse right away if you have any of these symptoms while getting one of these drugs.

Autoimmune reactions: These drugs work by basically removing one of the safeguards on the body's immune system. Sometimes this causes the immune system to attack other parts of the body, which can cause serious or even life-threatening problems in the lungs, intestines, liver, hormone-making glands, kidneys, skin, or other organs.

It's very important to report any new side effects to your health care team right away. If you do have a serious side effect, treatment may need to be stopped and you may be given high doses of corticosteroids to suppress your immune system.

More information about immunotherapy

To learn more about how drugs that work on the immune system are used to treat cancer, see Cancer Immunotherapy[1].

To learn about some of the side effects listed here and how to manage them, see Managing Cancer-related Side Effects[2].

Hyperlinks

1. www.cancer.org/cancer/managing-cancer/treatment-types/immunotherapy.html
2. www.cancer.org/cancer/managing-cancer/side-effects.html

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Treatment Choices for Endometrial Cancer, by Stage

The stage (extent) of endometrial cancer is the most important factor in choosing treatment. But other factors can also affect your treatment options, including the type of cancer, your age and overall health, and whether you want to be able to have children. Tests done on the cancer cells are also used to find out if certain treatments, like hormone, immunotherapy, and targeted therapy, might work.

Surgery is the first treatment for almost everyone with endometrial cancer. The operation includes removing the uterus, fallopian tubes, and ovaries. (This is called a **total hysterectomy/bilateral salpingo-oophorectomy** or TH/BSO). Lymph nodes from the pelvis and around the aorta may also be removed (a pelvic and para-aortic **lymph node dissection** [LND] or sampling) and tested for cancer spread. **Pelvic washings** may be done, too. The tissues removed at surgery are tested to see how far the cancer has spread (staging). Depending on the stage of the cancer, other treatments, such as radiation and/or chemotherapy may be recommended.

For some who still want to be able to get pregnant, surgery may be put off for a time and other treatments tried instead.

If a patient isn't well enough to have surgery, other treatments, like radiation, will be used.

Treating stage I endometrial cancers

Stage I is either only in the uterus or is a non-aggressive type limited to the uterus and ovary. It has not spread to lymph nodes or distant sites.

Stage I endometrioid cancers

Standard treatment includes surgery to remove and stage[1] the cancer. Sometimes this is the only treatment needed. The patient is then closely watched for signs that the cancer has come back (recurred).

For those with higher grade tumors, radiation will likely be recommended after surgery. Vaginal brachytherapy (VB), pelvic radiation, or both can be used.

Some younger patients with early endometrial cancer may have just the uterus removed without removing the ovaries. This prevents menopause and the problems that can come with it. This also increases the chance that the cancer will come back, but it doesn't make it more likely that you will die from the cancer. This may be something that you want to discuss with your doctor.

Those who cannot have surgery because of other medical problems or age are often treated with just radiation (external radiation and/or vaginal brachytherapy).

Fertility-sparing treatment for stage IA grade 1 endometrioid cancers

For those who still want to have children, surgery may be postponed while progestin therapy is used to treat the cancer. Progestin treatment can cause the cancer to shrink or even go away for some time, giving them a chance to get pregnant. Still, this can be risky if the patient isn't watched closely. An endometrial biopsy or a D&C should be done every 3 to 6 months. If there's still no cancer after 6 months, pregnancy can be attempted. Regular checks for the cancer will need to continue every 6 months. After childbearing is complete, a total hysterectomy with bilateral salpingectomy with or without bilateral oophorectomy is recommended because the cancer can come back again.

Progestin treatment often doesn't work and the cancer doesn't get better or keeps growing. Putting off surgery can give the cancer time to spread outside the uterus. If it doesn't go away in 6 to 12 months, surgery (hysterectomy and removal of both fallopian tubes and ovaries) to remove and stage the cancer is recommended.

A second opinion from a gynecologic oncologist and pathologist (to confirm the grade[2] of the cancer) before starting progestin therapy is important. Seeing a fertility expert is also a good idea. It's important to understand that this isn't a standard treatment and might increase risk of cancer growth and spread.

Other types of stage I endometrial cancers

Uterine cancers such as papillary serous carcinoma, clear cell carcinoma, or carcinosarcoma are more likely to have already spread when diagnosed. If the biopsy done before surgery shows a high-grade cancer, the surgery may be more extensive. Along with the total hysterectomy, both fallopian tubes, both ovaries, and lymph nodes are removed and the omentum is biopsied.

After surgery, chemotherapy (chemo) with or without radiation therapy is given to help keep the cancer from coming back. The chemo usually includes carboplatin and paclitaxel, but other drugs can also be used.

If the cancer can't be removed with surgery, chemotherapy (chemo) with or without radiation is used. Sometimes, the tumor then shrinks so that surgery can remove it.

Treating stage II endometrial cancers

When an endometrial cancer is stage II, it has either spread to the connective tissue of the cervix or there is substantial lymphovascular space invasion (LVSI; cancer cells spread to the blood vessels or lymph vessels) or the cancer is an aggressive histologic type with any invasion into the myometrium.

One treatment option is to have surgery first, followed by radiation therapy. If the cancer has spread to the cervix, the surgery may include a radical hysterectomy.

Radiation therapy, often both vaginal brachytherapy and external pelvic radiation, may be given after the patient has recovered from surgery. Another option is to give the radiation therapy first, and then do a simple hysterectomy, remove both ovaries and fallopian tubes (BSO), and possible lymph node dissection or sampling.

If there’s cancer in the lymph nodes that have been removed and checked for cancer cells, the cancer is actually stage IIIC.

Some patients with early-stage endometrial cancer might not be healthy enough to safely have surgery. These patients are treated with external radiation and brachytherapy.

For patients with high-grade cancers, like papillary serous carcinoma or clear cell carcinoma, surgery may include omental biopsy or omentectomy (removal of the omentum) and peritoneal biopsies along with the total hysterectomy, removal of both fallopian tubes and ovaries, assessment of the lymph nodes, and pelvic washings.

After surgery, radiation therapy, chemo, or both may be given to help keep the cancer from coming back. The chemo usually includes the drugs cisplatin or carboplatin and paclitaxel.

Someone with a stage II uterine carcinosarcoma often has the same type of surgery that's used for a high-grade cancer. After surgery, radiation or chemo, or both may be used.

Also included in stage II are cancers that are confined to the uterus, but molecular classification detects an abnormality in the p53 gene. This is designated stage IICmp53abn. Most endometrial cancers with an abnormality in the p53 gene are higher grade, but this classification includes lower grade tumors that are found to have this marker that is associated with worse outcomes than low-grade tumors that do not have a p53 gene abnormality.

Treating stage III endometrial cancers

Stage III endometrial cancers have spread outside of the uterus.

If the surgeon thinks that all visible cancer can be removed, a hysterectomy is done and both ovaries and fallopian tubes are removed. Sometimes those with stage III cancers need a radical hysterectomy. A pelvic and para-aortic lymph node dissection may also be done. Pelvic washings will be done, and the omentum may be removed. Some doctors will try to remove any remaining cancer (called debulking), but it isn't clear that this helps patients live longer.

If tests done before surgery[3] show that the cancer has spread too far to be removed completely, in rare cases, radiation therapy may be given before any surgery. It might shrink the tumor enough to make surgery an option.

Stage IIIA endometrial cancer

The cancer has spread to the ovary or fallopian tube and may have spread to the outer surface of the uterus (called the **serosa**). For these cancers, treatment after surgery may include chemo, radiation, or both. Radiation is given to the pelvis or to both the abdomen (belly) and pelvis. Vaginal brachytherapy is often used, too.

Stage IIIB endometrial cancer

The cancer has spread to the vagina or to the tissues around the uterus (called the parametrium). It may have also spread to the tissues within the pelvis (called the pelvic peritoneum). After surgery, stage IIIB may be treated with chemo and/or radiation.

Stage IIIC endometrial cancer

This includes cancers that have spread to the lymph nodes in the pelvis (stage IIIC1) and those that have spread to the lymph nodes around the aorta (stage IIIC2). Treatment includes surgery, followed by chemo and/or radiation.

For those with high-grade cancers (such as papillary serous carcinoma, clear cell carcinoma, or carcinosarcoma), the surgery may include omentectomy and peritoneal biopsies along with a total hysterectomy, removal of both ovaries and fallopian tubes, pelvic and para-aortic lymph node dissections, and pelvic washings. After surgery, chemo or radiation therapy, or both may be given to help keep the cancer from coming back. The chemo usually includes cisplatin or carboplatin and paclitaxel.

Treating stage IV endometrial cancers

Stage IVA endometrial cancers have grown into the bladder or bowel.

Stage IVB endometrial cancers have spread to the abdominal peritoneum.

Stage IVC The cancer has spread to distant sites such as lungs, liver, brain or bone or has spread to lymph nodes outside of the abdomen or lymph nodes above the kidneys.

Some endometrial cancers are stage IV because they have spread to lymph nodes in the abdomen (and not just the pelvis and para-aortic area), but they haven't spread to any other areas. Patients with this kind of cancer spread may have better outcomes if all the cancer that's seen can be removed (debulked) and biopsies of other areas in the abdomen do not show cancer cells.

In many cases of stage IV endometrial cancer, the cancer has spread too far for it all to be removed with surgery. A hysterectomy and removal of both fallopian tubes and ovaries may still be done to prevent excessive bleeding. Radiation therapy may also be used for this reason. When the cancer has spread to other parts of the body, hormone therapy may be used. But high-grade cancers and those without detectable progesterone and estrogen receptors on the cancer cells are not likely to respond to hormone therapy.

Combinations of chemo drugs may help some patients for a time. Targeted drugs and /or immunotherapy drugs may also be options for some with advanced endometrial cancer.

Those with stage IV endometrial cancer should consider taking part in clinical trials[4] of chemotherapy or other new treatments.

Treating recurrent endometrial cancer

A cancer that comes back after treatment is called a **recurrence**. Recurrence can be local (in or near the same place it started) or distant (spread to organs such as the lungs or bone). Treatment depends on the amount of cancer and where it is, as well as the treatment that was used the first time.

For **local** recurrences, such as in the pelvis, surgery (sometimes followed by radiation therapy) is used. For those with other medical conditions that make them unable to have surgery, radiation therapy alone or combined with hormone therapy tends to be used.

For **a distant** (in other parts of the body) recurrence, surgery and/or focused radiation therapy may be used when the cancer is only in a few small spots (like in the lungs or bones). Those with more extensive recurrences (widespread cancer) are treated like those with stage IV endometrial cancer. Low-grade cancers containing hormone receptors are more likely to respond well to hormone therapy. Higher-grade cancers are unlikely to shrink with hormone therapy but may respond to chemo, targeted therapy and immunotherapy.

Clinical trials[5] should be considered and may be a good option.

Hyperlinks

1. www.cancer.org/cancer/types/endometrial-cancer/detection-diagnosis-staging/staging.html
2. www.cancer.org/cancer/types/endometrial-cancer/about/what-is-endometrial-cancer.html
3. www.cancer.org/cancer/types/endometrial-cancer/detection-diagnosis-staging/how-diagnosed.html
4. www.cancer.org/cancer/managing-cancer/making-treatment-decisions/clinical-trials.html
5. www.cancer.org/cancer/managing-cancer/making-treatment-decisions/clinical-trials.html

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